

Ultragenyx (NASDAQ: RARE) — UX111 / Sanfilippo Syndrome A

PDUFA Diligence Memo — AAV gene therapy for MPS IIIA · FDA action date Sept 19, 2026

Signal OS · public_biotech vertical · dated 2026-06-02 · RARE \$21.79 · **regulatory/CMC catalyst (not a clinical binary)** · not investment advice

Verdict

PoS of approval (Sept-19 PDUFA): ~75–85%. The efficacy/surrogate question is already settled in the company's favor; the only open issue is manufacturing (CMC).

The July-2025 CRL was CMC-ONLY — it did not touch the clinical data, safety, or the CSF-HS surrogate. A second CRL would again be a delay, not a death.

The entire PoS hinges on one UNVERIFIABLE item: the Andelyn/Bedford manufacturing re-inspection. FDA accepting the resubmission + Priority Review is a strong positive signal but does not confirm the facilities cleared.

Best risk/reward in the RARE complex — high-PoS, near-term, asymmetric on a beaten-down stock — but a de-risking/validation catalyst, not a transformational one.

1 · Snapshot

Catalyst	PDUFA action date Sept 19, 2026 (Priority Review) on the resubmitted BLA; accelerated approval pathway.
Asset	UX111 (ABO-102) — one-time AAV9 gene therapy for Sanfilippo Syndrome Type A (MPS IIIA) , a fatal pediatric neurodegenerative lysosomal disorder. Would be first-ever therapy in the disease.
Basis	Accelerated approval on CSF heparan sulfate surrogate (FDA-agreed); TRANSPHER A: 65% median CSF-HS reduction + statistically significant Bayley-III cognition/communication gains vs natural history.
Regulatory history	BLA Dec-2024 → CRL Jul-11-2025 (CMC/manufacturing only) → resubmitted → FDA accepted Apr-2-2026 (Class 2, 6-mo review).
RARE	\$21.79, ~\$2.0B cap, ~50% off highs (post-setrusumab Dec-2025 failure); IV ~60% (one driver, not the whole story).

2 · R / f(M) Diligence — Manufacturing → PDUFA

Each row: R = claim & source; f = verification method; Finding = AFFIRMED / UNVERIFIABLE / CONDITIONAL + evidence.

#	R — claim & method (f)	Finding
1	Efficacy/surrogate settled. FDA agreed CSF-HS is a reasonable surrogate; 65% CSF-HS reduction + sig. Bayley-III vs natural history. (f: cross-check FDA agreement + clinical PRs vs CRL scope)	AFFIRMED — surrogate accepted; the CRL explicitly did NOT touch clinical. Efficacy is de-risked (load-bearing positive).
2	CRL scope. The Jul-11-2025 CRL was CMC/manufacturing only. (f: read CRL announcement)	AFFIRMED — verbatim: “did not note any review issues related to the clinical data package nor clinical inspections.”
3	Nature of CMC issues. Observations are “facilities and processes, not directly related to product quality.” (f: CRL language)	AFFIRMED (company-framed) — FDA's 483 observations are not public, so the depth is UNVERIFIABLE .
4	Resubmission + acceptance. Accepted Apr-2-2026; Priority Review; PDUFA Sept-19-2026. (f: verify acceptance + date)	AFFIRMED (multiple sources).
5	Resubmission class. ~6-month clock ⇒ Class 2 (major amendment). (f: PDUFA timing math)	AFFIRMED (inferred) — substantive CMC changes requiring full review. Neutral.
6	Manufacturing sites. Andelyn Biosciences (Columbus, OH) + Ultragenyx (Bedford, MA), all US. (f: confirm facilities)	AFFIRMED — these are the inspected sites; their re-inspection gates the decision.

#	R — claim & method (f)	Finding
7	Advisory committee. No AdComm planned. (f: FDA calendar + PRs)	AFFIRMED for original BLA ; not explicitly reconfirmed for the resubmission. Likely none (CMC-only), but not fully verified.
8	Re-inspection status. Has FDA re-inspected & cleared the Andelyn/Bedford observations? (f: search FDA inspection / 483-closure records)	UNVERIFIABLE — no public confirmation. FDA acceptance ≠ facility cleared. THE single residual risk the PoS hinges on.
9	PRV. Rare-Pediatric-Disease voucher (~\$100–150M) on approval. (f: confirm RPD designation + program status)	CONDITIONAL — RPD likely designated, but the PRV program's designation authority sunset (Dec-2024) makes issuance program-dependent. UNVERIFIABLE without the cutoff.
10	Sector context. AAV gene-therapy CMC is a known FDA pain point (field-wide manufacturing CRLs). (f: sector precedent)	AFFIRMED context — raises the bar on the re-inspection; not UX111-specific.

3 · Diligence Conclusion

- **The hard half is de-risked** — efficacy, safety, and the surrogate are settled (R1, R2). That is why this is a high-PoS catalyst, not a clinical binary.
- **The PoS rests on one unverifiable item — the manufacturing re-inspection (R8).** Priority-Review acceptance is a strong positive signal but does not confirm Andelyn/Bedford cleared. A second CRL would again be CMC, not clinical — a delay.
- **PoS of approval ~75–85%;** swing factor = the re-inspection. CMC-only CRLs resubmitted with Priority Review resolve at a high rate, tempered by AAV manufacturing being a sticky FDA area.

4 · Valuation (separate from the R/f(M) pipeline)

The UX111 asset: ultra-rare (~40 US new cases/yr; small prevalent treatable pool, benefit only in young pre-degeneration patients); one-time gene therapy ~\$3–4M → a bolus then steady-state; **peak ~\$150–400M (lumpy)**; risk-adjusted asset value **~\$0.5–1.0B** + a PRV ~\$100–150M (conditional).

PDUFA outcome	Prob	Stock impact on RARE (\$21.79, ~\$2.0B, beaten-down)
Approval	~80%	De-risks asset + PRV + removes a key overhang / validates pipeline post-setrusumab. Partly priced → ~+10 to +20%
Second CRL (CMC)	~20%	Another delay (not death) + sentiment blow on a depressed stock → ~-10 to -25%

Synthesis: high-PoS (~80%), near-term, asymmetric on a beaten-down stock where even the downside is a *delay* on an asset whose science is already cleared — the best risk/reward in the RARE complex. But it is a **de-risking / validation catalyst, not a transformational one**: Sanfilippo is a small market (~\$0.5–1B asset, ~25–50% of cap), so it re-rates sentiment and removes an overhang more than it transforms fundamental value. The bigger swing for RARE is the combination (UX111 approval + base business + GTX-102 optionality) at a discounted ~\$2B.

5 · Open Items to Close

- **Andelyn/Bedford re-inspection (R8)** — the swing on the ~80%. Watch for an Ultragenyx update or an FDA inspection record.
- **PRV cutoff (R9)** — whether UX111's RPD designation still yields a voucher post-sunset (~\$100–150M).

Sources: Ultragenyx IR (CRL Jul-11-2025; resubmission acceptance Apr-2-2026; PDUFA Sept-19-2026); Cure Sanfilippo Foundation (CRL scope); CGTLive / NeurologyLive (resubmission, Priority Review); FiercePharma (AAV manufacturing context); TRANSPHER A (NCT02716246) clinical data. Live RARE quote via IBKR (read-only).

Disclaimer: This is regulatory/microstructure research, not investment, legal, or tax advice. PoS and valuation figures include estimates and items explicitly marked UNVERIFIABLE/CONDITIONAL; verify before acting. A PDUFA is a regulatory decision, distinct from the honesty-screen framework used for trial readouts.